
WINAMOL CF Tablets

Name and Strength of Active Ingredient:

Each tablet contains:

Paracetamol	325mg
Pseudoephedrine Hydrochloride.....	30mg
Chlorpheniramine Maleate.....	2mg

Product Description:

White to off-white colour, circular biconvex tablet with debossing “CF” on top and plain on the bottom.

Pharmacodynamics:

Paracetamol is a para-aminophenol derivative that exhibits analgesic and anti-pyretic activity. It does not possess anti-inflammatory activity. It is given by mouth or rectally for mild to moderate pain and fever.

Pseudoephedrine hydrochloride is a direct and indirect acting sympathomimetic agent. It is a stereoisomer of ephedrine and has a similar action but has less pressor activity and central nervous system effects. It is administered orally for symptomatic relief from nasal congestion.

Chlorphenamine maleate is an antihistamine used for relief from hypersensitivity reactions which also has sedative properties.

Pharmacokinetics:

Paracetamol is readily absorbed from the gastrointestinal tract with peak plasma concentrations occurring about 10 to 60 minutes after oral administration and is distributed into most body tissues. The elimination half-life of paracetamol varies from about 1 to 3 hours. It is metabolized predominantly in the liver and excreted in the urine mainly as the glucuronide and sulphate conjugates, less than 5% is excreted as unchanged paracetamol.

Pseudoephedrine is readily absorbed from the gastrointestinal tract. It is resistant to metabolism by monoamine oxidase and is largely excreted unchanged in the urine together with small amounts of its hepatic metabolite. It has a half-life of about 5 to 8 hours; elimination is enhanced and half-life accordingly shorter in acid urine. Small amounts are distributed into breast milk.

Chlorpheniramine maleate is absorbed relatively slowly from the gastrointestinal tract, peak plasma concentrations occurring about 2.5 to 6 hours after oral administration. It appears to undergo considerable first-pass metabolism. About 70% of chlorpheniramine in the circulation is bound to plasma proteins. Values ranging from 2 to 43 hours have been reported for the half-life. Chlorpheniramine maleate is widely distributed in the body and is extensively metabolised. Metabolites include desmethyl- and didesmethylchlorpheniramine. Unchanged drug and metabolites are excreted primarily in the urine; trace amounts found in the faeces.

Indications:

For the symptomatic relief of headache, muscular ache, fever, sneezing, nasal congestion, and itching and watery eyes, associated with flu, common colds and allergic rhinitis.

Recommended Dose:

For oral administration.

Minimum dosing interval: 4 hours.

Do not exceed the stated dose or frequency of dosing.

Should not be used with other products containing paracetamol, decongestant or antihistamines, including cough and cold preparations.

Adults (including the elderly) and children 12 years and over:

Take 1 to 2 tablets every 4 to 6 hours as needed. Maximum daily dose: 8 tablets in 24 hours. Maximum duration of continued use without medical advice: 7 days.

Minimum dosing interval: 4 hours

Children aged 6-11 years:

Take 1 tablet every 4 to 6 hours as needed. Maximum daily dose: 4 tablets in 24 hours. Maximum duration of continued use without medical advice: 3 days.

Minimum dosing interval: 4 hours

Not recommended for children under the age of 6 years, except under medical advice.

Route of Administration:

Oral

Contraindications:

This product is contraindicated in patients:

- With a previous history of hypersensitivity to paracetamol, pseudoephedrine, chlorphenamine or other antihistamines.
- With severe hypertension or severe coronary artery disease
- Who are receiving other sympathomimetics (such as decongestants, appetite suppressant and amphetamine-like psychostimulants)
- Who are receiving Monoamine Oxidase Inhibitors (MAOIs), or for two weeks after stopping a MAOI drug
- With severe renal impairment

Warnings and Precautions:

Caution should be exercised in patients with liver impairment, or mild to moderate kidney impairment. Underlying liver disease increases the risk of paracetamol-related liver damage.

Caution should be exercised in patients with cardiovascular disease, arrhythmias, hypertension, hyperthyroidism, prostatic enlargement, diabetes, glaucoma, or pheochromocytoma, epilepsy, bronchitis, bronchiectasis and bronchial asthma.

The anticholinergic properties of chlorphenamine may cause drowsiness, dizziness, blurred vision and psychomotor impairment in some patients which may seriously affect ability to drive and use machinery.

Chlorphenamine may increase the effects of alcohol and therefore concurrent use should be avoided.

Concurrent use with drugs which cause sedation, such as anxiolytic and hypnotics may cause an increase in sedative effects; therefore medical advice should be sought before taking chlorphenamine concurrently with these medicines.

Use with caution in patients taking beta-blockers or other anti-hypertensives (see Drug Interactions).

Children and elderly are more likely to experience neurological anticholinergic effects and paradoxical excitation (e.g. increased energy, restlessness, nervousness).

The anticholinergic properties of chlorphenamine may cause drowsiness, dizziness, blurred vision and psychomotor impairment in some patients which may seriously affect ability to drive and use machinery.

If symptoms persist, medical advice should be sought.

Keep out of reach of children.

When used for treatment of cough and cold:

(a) Not to be used in children less than 2 years of age

(b) To be used with caution and doctor's/ pharmacist's advice in children 2 to 6 years of age.

This preparation contains PARACETAMOL. Do not take any other paracetamol containing medicines at the same time.

Allergy alert: Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters, or rash. These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

Interactions with Other Medicaments:

The anticoagulant effect of warfarin and other coumarins may be enhanced by prolonged regular daily use of paracetamol-containing products with increased risk of bleeding; occasional doses have no significant effect.

Concomitant administration of pseudoephedrine hydrochloride-containing products and MAOIs (or within two weeks of stopping MAOI) may lead to hypertensive crisis. The anticholinergic effects of chlorphenamine are intensified by MAOIs.

Concomitant use of this medication with sympathomimetic agents (such as decongestant, appetite suppressant and amphetamine-like psychostimulants) which interfere with the catabolism of sympathomimetic amines may occasionally cause a rise in blood pressure (see Contraindications).

Pseudoephedrine-containing products may antagonize the effect of certain classes of antihypertensive (e.g. beta blockers, methyl-dopa, reserpine, debrisoquine, guanethidine).

Concurrent use of chlorphenamine and hypnotics or anxiolytics may potentiate drowsiness. Concurrent use of alcohol may have similar effect.

Chlorphenamine inhibits phenytoin metabolism and can lead to phenytoin toxicity.

Pregnancy and Lactation:

Pregnancy

This product should not be used in pregnancy unless considered essential by a physician.

Safe use of pseudoephedrine in pregnancy has not been established despite widespread use over many years. Caution should therefore be exercised by balancing the potential benefit of treatment to the mother against any possible hazards to the developing foetus.

There are no adequate data from the use of chlorphenamine maleate in pregnant women. The potential risk for human is unknown. Use during the third trimester may result in reactions in the newborn or premature neonates.

Lactation

This product should not be used whilst breastfeeding unless considered essential by a physician.

Pseudoephedrine is excreted in breast milk in small amounts but the effect of this on breast fed infants is unknown.

Chlorphenamine maleate and other antihistamines may inhibit lactation and may be secreted in breast milk.

Side Effects:

The following convention has been utilized for the classification of undesirable effects: very common ($\geq 1/10$), common ($\geq 1/100$, $< 1/10$), uncommon ($\geq 1/1,000$, $< 1/100$), rare ($\geq 1/10,000$, $< 1/1,000$), very rare ($< 1/10,000$), not known (cannot be estimated from the available data).

Adverse events frequencies have been estimated from spontaneous reports received through post-marketing data.

Paracetamol

Adverse events from historical clinical trial data are both infrequent and from small patient exposure. Accordingly, events reported from extensive post-marketing experience at therapeutic/labeled dose and considered attributable are tabulated below by System Organ Class and frequency.

Body System	Undesirable Effects	Frequency
Blood and lymphatic system disorders	Thrombocytopaenia	Very rare
Immune System disorders	Anaphylaxis	Very rare
	Cutaneous hypersensitivity reactions including skin rashes, angioedema, and Stevens Johnson syndrome	

Respiratory, thoracic and mediastinal disorders	Bronchospasm in patients sensitive to aspirin and other NSAIDs	Very rare
Hepatobiliary disorders	Hepatic dysfunction	Very rare

Pseudoephedrine

Body System	Undesirable Effects	Frequency
Psychiatric disorders	Nervousness, Insomnia	Common
	Agitation, restlessness	Uncommon
	Hallucinations	Rare
Nervous system disorders	Dizziness	Common
Cardiac disorders	Tachycardia, palpitations	Rare
Vascular disorders	Increased blood pressure*	Rare
Gastrointestinal disorders	Vomiting, dry mouth, nausea	Common
Skin and subcutaneous tissue disorders	Rash, allergic dermatitis**	Rare
Renal and urinary disorders	Dysuria, urinary retention***	Uncommon

*Increased in systolic blood pressure have been observed. At therapeutic doses, the effects of pseudoephedrine on blood pressure are not clinically significant.

**A variety of allergic skin reactions, with or without systemic features such as bronchospasm and angioedema have been reported following use of pseudoephedrine.

***Urinary retention is most likely to occur in those with bladder outlet obstruction, such as prostatic hypertrophy.

Chlorphenamine

Adverse reactions which have been observed in historical clinical studies and which are considered common or very common are listed below by MeDRA System Organ Class. The frequency of other adverse reactions identified during post-marketing use is unknown.

Body System	Undesirable effect	Frequency
Immune system disorders	Allergic reactions, angioedema, anaphylactic reactions	Unknown
Metabolism and nutritional disorders	Anorexia	Unknown
Psychiatric disorders	Confusion*, excitation*, irritability*, nightmares*	Unknown
Nervous system disorders	Sedation, somnolence	Very common
	Disturbance in attention, abnormal coordination, dizziness, headache	Common
Eye disorders	Blurred vision	Common
Vascular disorders	Hypotension	Unknown
Respiratory, thoracic and mediastinal disorders	Thickening of bronchial secretions	Unknown
Gastrointestinal disorders	Nausea, dry mouth	Common
	Vomiting, abdominal pain, diarrhea, dyspepsia	Unknown
Skin and subcutaneous disorders	Exfoliative dermatitis, rash, urticaria, photosensitivity	Unknown
Musculoskeletal and connective tissue disorders	Muscle twitching, muscle weakness	Unknown
Renal and urinary disorders	Urinary retention	Unknown
General disorders and administration site conditions	Fatigue	Common
	Chest tightness	Unknown

*Children and elderly are more susceptible to neurological anticholinergic effects and paradoxical excitation (e.g. increased energy, restlessness, nervousness)

Symptoms and Treatment of Overdose:

Paracetamol

Sign and symptoms

Paracetamol overdose may cause liver failure.

Treatment

Immediate medical management is required in the event of overdose, even if symptoms of overdose are not present. Administration of N-acetylcysteine or methionine may be required.

Pseudoephedrine Hydrochloride

Sign and symptoms

Pseudoephedrine overdose may results in symptoms due to central nervous system and cardiovascular stimulation e.g. excitement, restlessness, hallucinations, hypertension and arrhythmias. In severe cases, psychosis, convulsion, coma and hypertensive crisis may occur. Serum potassium levels may be low due to extracellular to intracellular shifts in potassium.

Treatment

Treatment should consist of standard supportive measures. Beta blockers should reverse the cardiovascular complication and the hypokalemia.

Chlorphenamine Maleate

Sign and symptoms

Overdose is likely to result in effects similar to those listed under adverse reactions. Additional symptoms may include paradoxical excitation, toxic psychosis, convulsions, apnoea, dystonic reactions and cardiovascular collapse including arrhythmias.

Treatment

Treatment should be supportive and directed towards specific symptoms. Convulsions and marked CNS stimulation should be treated with parenteral diazepam.

Effects on Ability to Drive and use Machine:

This product may cause drowsiness, dizziness or blurred vision. You should not drive or operate machinery until you are sure you are not affected.

Preclinical Safety Data:

Not Applicable

Instructions for Use:

For oral use only.

Storage Condition:

Store below 30°C. Protect from light.

Pack Size:

Packed in Alu-PVDC strip. Each strip contains 10 tablets. The blisters are packed into 1 x 10's and 10 x 10's in a pre-printed carton along with a package insert.

Registration Number:

MAL23036011AZ

Manufactured by & Product Registration Holder:

KCK Pharmaceutical Industries Sdn. Bhd.

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